



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ماجده ابراهيم رمضان ابراهيم

العنوان : =

الحي / المدينة : ديرب نجم

المحافظة : الشرقية

تاريخ تسجيل الإستمارة : 03-10-2026

رقم البطاقة : 27510011306161 رقم التليفون : 01143617703

العمر (تاريخ الميلاد) : 01-10-1975 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ماجده ابراهيم رمضان ابراهيم National ID: 27510011306161	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

ماجده
ابراهيم رمضان ابراهيم
شبرا صوره
مركز ديرب نجم - الشرقية

١٩٧٥/١٠/٢١ ٢٧٥ ١٠ ٠١ ١٣ ٠٦ ٦١

LK8592151

٢٠٢٥/١٠ ٢٧٥١٠٠١١٣٠٦١٦١

حاصله على دبلوم تجارة

أنثى مسلمة متزوجة
مصطفى عبده امام جبر

البطاقة سارية حتى ٢٠٣٢/١٠/٢٧

٨

Patient Name: Magda Ibrahim Ramadan Ibrahim	Date : 09/10/2025
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 4000/2021	Pat. ID: 25494

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
- UOQ, architectural distortion with no speculations, masses, asymmetry , or suspicious microcalcifications.
- Ultrasound demonstrated UOQ area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): Post-therapeutic sequelae.

Left breast (BIRADS 1): Normal study.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Name: <u>ماجده ابراهيم رمضان ابراهيم</u>		
MR. : <u>4072</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient underwent upfront radiation therapy and wide local excision (WLE) combined with AC chemotherapy in October 2021 (PT1 PN1 1/13). An adjusted chemotherapy regimen was administered with placement of a port. Pathology demonstrated estrogen receptor positive, progesterone receptor positive, HER2 negative disease with a Ki-67 index of 4%. The patient has been on an aromatase inhibitor since 2022 for breast cancer management. No further cycle or week markers were documented; the current status and plan are as recorded. The patient continues femara (letrozole) together with supplemental therapy for more than three months. Zoledronic acid (Zometa) was administered today, and calcium supplementation (dose 1,2) was ordered through the pharmacy. No explicit cycle or week counts are noted; the current status and plan are as documented.		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	
	هنا اسامه عبد الرحيم عبد البر	الطبيب المعالج :
يعتمد ,		

Quick

Name	Mrs. Magda Ibrahim Ramadan Ibrahim		
Ref. By Prof. Dr.	Hisham Al-Ghazaly	Date	21/9/2021
ID	6621014838	Acc. No.	8464353

¹⁸F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of right breast IDC (pathology 11 / 09 / 2021), under-investigation.

Total body PET scan was done 40 minutes after IV administration of 300 MBq of ¹⁸F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROIs & the normal threshold would be the normal blood pool liver SUV < 2 .

Reported By:

Dr. Islam Aboulwafa

Approved By:

Prof. Dr.

Essam El-Shiekh

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.
- Normal naso-/oro-pharynx, larynx and salivary glands.
- *Non homogenous thyroid gland, for further thyroid work-up.*
- No PET/CT abnormal (enlarged FDG hypermetabolic) cervical L.Ns seen.

Torso:

Thorax:

- *Small right breast outer domain ± 1.5 cm nodule of faint increased activity eliciting $SUV_{max} 2.2$.*
- *Small right axillary lymph nodes, largest ± 1.2 cm eliciting $SUV_{max} 1.7$.*
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No PET/CT abnormal (enlarged FDG hypermetabolic) hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.

Abdomen & Pelvis:

- *The liver is mildly enlarged with no FDG hypermetabolic focal lesions.*
- No intra-/extra-hepatic biliary radicals dilatation.
- Normal appearance of spleen, pancreas, adrenals and pelvic organs. (Bulky uterus likely fibroid; however, for Trans-vaginal ultrasound assessment).

②5112

Name	Mrs. Magda Ibrahim Ramadan Ibrahim		
Ref. By Prof. Dr.	Hisham Al-Ghazaly	Date	21/9/2021
ID	6621014838	Acc. No.	8464353

- No PET/CT abnormal (enlarged FDG hypermetabolic) abdominal or pelvic L.Ns seen.
- Normal physiological biodistribution over the bowel & urinary tracts.

Surveyed skeleton:

- *Diffuse increased bone marrow uptake SUV_{max} 3.5, yet no underlying lesions, mostly hematopoietic.*
- No tracer active lytic or sclerotic lesions seen.

Opinion:

- Considering the provided histogenic data, right breast carcinoma with axillary nodal metastatic deposits, for mammographic correlation.
- Rest of the surveyed body is free from any other FDG hypermetabolic neoplastic lesions.

Thanks for your referral;

Dr. Ahmed Kandeel
MD Nuclear Medicine

Dr. Islam Aboulwafa, M.Sc.
Prof. Dr. Essam Ali El-Shiekh

Patient Name: Magdah Abraham Ramdan	Study Date: 22/09/2021
Ref. Phy.: Hesham El-ghazali	Patient ID: 521483

**BILATERAL DIGITAL CONTRAST ENHANCED COMPUTED SONO-
MAMMOGRAPHIC EXAMINATION:-**

Technique:

- Bilateral CC and MLO view injection omniopaque 1.5 mg / kg.

Findings:

- Both breasts show normal nipple, areola, skin and subcutaneous tissue thickness.
- Both breasts show predominant glandular tissue with coarse nodularity impressive of fibroadenosis.
- No malignant micro calcifications are detected at either breast.
- **Sonographic examination** confirmed the mammographic findings and revealed a hypoechoic soft tissue lesion seen at right 9 O'clock position measuring 12 X 6 mm.
- Bilateral axillary mildly enlarged lymph nodes, mostly reactive.
- Mammogram contrast revealed no evidence enhancement could be detected at that lesion.

CONCLUSION:-

- Right breast suspicious soft tissue lesion (BIRADS-6) which proved by pathological correlation.

MUCH OBLIGED

A. PROF. DR. SAFIENAZ YOUSSEF

PROF. DR. MOHAMED ABO ELHODA

Patient Name : الأستاذة / ماجدة إبراهيم رمضان إبراهيم
Patient No. : 27510011306161
Path No. : 2559/21
Sex : Female
Age : 46 Y
Referring Doc :

Request No. : 132666
Request Date : 26/10/2021
Verification Date : 09/11/2021
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient

Pathology Report

Clinical Data: Breast lesion

Nature of the specimen: wider excision

Gross Examination: wider excision measure 6x5x1.6cm, skin 9x1cm cut section revealed grayish white rubbery tissue

- Procedure: wider excision
- Localization: Right

Microscopic examination:

- Histological type: Resection site changes with only one duct of low grade DCIS, free extra margins

Diagnosis:

Right breast, Wider excision;

- Resection site changes with only one duct of low grade ductal carcinoma in situ
- Free extra margin.

Prof. Dr. Amal Hareedy
Dr. Mona Mohamed

Patient Name : الأستاذ / ماجده ابراهيم رمضان ابراهيم	Request No. : 138610
Patient No. : 27510011306161	Request Date : 05/10/2021
Path No. : 2335/21	Verification Date : 01/01/2023
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 46 Y	Department : Out Patient
Referreing Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer

Nature of the specimen: Wide local excision and axillary lymph nodes
Gross Examination: WLE measure 8x6x3 cm, covering skin 5x2.5 cm, axillary fat measure 9x3 cm
• Procedure: WLE and axillary lymph nodes
• Localization: Right
• Tumor extent
o Number: One
o Size: 1.5x1.5 cm
o Skin involvement: No
• Distance from margins: 1cm from deep margin
• Lymph nodes number and size: 13 LNs, up to 2cm in diameter
• Additional pathologic findings: N/A
• Immunohistochemistry technique: N/A

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
• Histological type: Invasive duct carcinoma.
• Grade: II
• Angioinvasion: No
• Ductal carcinoma in situ: yes (70%)
• Lymph node involvement: 1/13
• Resection Margins: the superior margin showed ductal carcinoma insitu, the other margins were free.
o Free /DCIS/ invasive carcinoma

Verified by :
dr.esraa magdy

Patient Name الأستاذة / ماجده ابراهيم رمضان ابراهيم
Patient No. 27510011306161
Path No. 2335/21
Sex Female
Age 46 Y
Referring Doc.

Request No. 138610
Request Date 05/10/2021
Verification Date 01/01/2023
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

- Histologic response to therapy: N/A
- Additional pathologic findings: N/A

-IPT:

ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
Her-2: No membrane reaction in tumor cells (score 0)
Ki 67: 20 % nuclear reaction in tumor cells
Positive control showed appropriate reactivity

Diagnosis:

Right breast, Wide local excision and axillary lymph nodes;

• Invasive ductal carcinoma, grade II
Major DCIS component 70%

• No vascular emboli.

Superior margin was involved by ductal carcinoma insitu.

• Nipple and areola and other margins are free.

o LNs 1/13

o pT1 pN1

-IPT:

ER: Positive (score 8/8).

PR: Positive (score 8/8).

HER-2: Negative (score 0).

Ki67: Positive in 20% of tumor cells.

Prof. Dr. Amal Hareedy

Prof. Dr. Gamal Abdelatty

Verified by :
dr.esraa magdy

Dr. Mona Mohamed

Dr. Esra

Printing Date : 02/01/2023 13:37:44

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Printed By : .marwa mohamed youss

صحة المرأة



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي



Patient Name الأستاذة / ماجده ابراهيم رمضان ابراهيم
Patient No. 27510011306161
Path No. 2335/21
Sex Female
Age 46 Y
Referreing Doc.

Request No. 138610
Request Date 05/10/2021
Verification Date 01/01/2023
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

magdy

Verified by :
dr.esraa magdy

Printing Date : 02/01/2023 13:37:44

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مركز الأورام (4) - الطابق الأول - مبنى رقم (987) - مستشفى عين شمس - مصر
Room No. (4) - First Floor-Building No. 987- Mina Corniche - Min El Qadima- Cairo, Egypt

Patient Name : الأستاذ / ماجده إبراهيم رمضان إبراهيم	Request No. : 138610
Patient No. : 27510011306161	Request Date : 05/10/2021
Path No. : 2335/21	Verification Date : 11/10/2021
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 46 Y	Department : Out Patient
Referring Doc :	

Pathology Report

Clinical Data: Right breast cancer

Nature of the specimen: Wide local excision and axillary lymph nodes

Gross Examination: WLE measure 8x6x3 cm, covering skin 5x2.5 cm, axillary fat measure 9x3 cm

- Procedure: WLE and axillary lymph nodes
- Localization: Right
- Tumor extent
 - o Number: One
 - o Size: 1.5x1.5 cm
 - o Skin involvement: No
- Distance from margins: 1cm from deep margin
- Lymph nodes number and size: 13 LNs, up to 2cm in diameter
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade: II
- Angioinvasion: No
- Ductal carcinoma in situ: yes (70%)
- Lymph node involvement: 1/13
- Resection Margins: the superior margin showed ductal carcinoma insitu, the other margins were free.
 - o Free /DCIS/ invasive carcinoma
- Histologic response to therapy: N/A
- Additional pathologic findings: N/A

Diagnosis:

- Right breast, Wide local excision and axillary lymph nodes;
- Invasive ductal carcinoma, grade II

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Page 1 of 2

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Patient Name : الأستاذة / ماجدة ابراهيم رمضان ابراهيم
Patient No. : 27510011306161
Path No. : 2335/21
Sex : Female
Age : 46 Y
Referring Doc :
Request No. : 138610
Request Date : 05/10/2021
Verification Date : 11/10/2021
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient

Pathology Report

- Major DCIS component 70%
- No vascular emboli.
- Superior margin was involved by ductal carcinoma insitu.
- Nipple and areola and other margins are free.
- o LNs 1/13
- o pT1 pN1

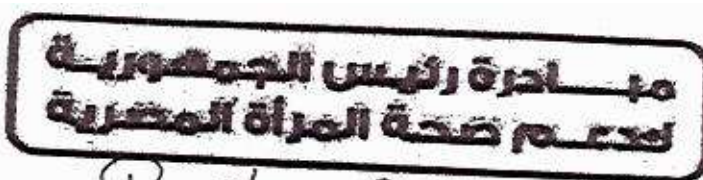
Prof. Dr. Amal Hareedy

Dr. Mona Mohamed

Printing Date : 12/10/2021 12:41:12

Page 2 of 2

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من إدارة رئيس الجمهورية
للدعم صحة المرأة المصرية

Post
MDT FORM

5701-112,717
11427177.2

<u>Name</u>	ماجدة ابراهيم رفاعه
<u>Age</u>	46
<u>MRN</u>	3000
<u>ID</u>	3000
<u>Co morbidity</u>	

post-operative

Clinical Data

Clinical Data	
Bx: Wider Excision	SM:
Free Extomargin, only one duct of	DCIS
IHC:	Mets Workup:
ER:	
PR:	
Her2:	previous pathology: superior
Ki67:	margin was T, N, M involved by DCIS
COMMENT:	LNs: 1/13
ER: 8/8	Her2 neu: -ve
PR: 7/8	Ecto
Ki 67: 4 %	

MDT OUTCOME & PLAN

For Adjuvant chemotherapy

FCC X3 $\rightarrow$ TXT X3 $\Rightarrow$ Followed by ATZ (switch)

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
J. S. M. 11/2	[Signature]	[Signature]	

5. 51 10/11/11

Day 11-11-21 E



حامد محمد

Patient Name: Magda Ibrahim Ramadan Ibrahim	Date : 04/01/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 48 Year(s)
Medical ID: 4000/2021	Pat. ID: 25494

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Engy Abdallah

04/01/2024 12:24 PM



Patient Name: Magda Ibrahim Ramadan Ibrahim	Date : 18/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 49 Year(s)
Medical ID: 4000/2021	Pat. ID: 25494

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2).
- Normal study of the left breast (BIRADS 1).

Signature Dr. Nehal Hamdy

18/12/2024 12:21 PM